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CARE HOSPITALS

DEPARTMENT OF ORTHOPAEDICS

DISCHARGE SUMMARY

UHID	: 80417321	IP NO	: BBIP26/60290
Name	: Mr SACHIN RAMESH MOHAPATRA	Bed No	: 620-A
Age/Gender	: 26 Years/Male	Admission Date	: 21/01/2026 10:49 AM
Primary Consultant	: Dr SANDEEP SINGH	Date Of Discharge	: 23/01/2026 10:30 AM
Discharge Type	: NORMAL	Patient Mobile	: 8993857127

DISCHARGE TYPE:DOCTOR ADVICE

PRIMARY CONSULTANT: Dr SANDEEP SINGH | SPECIALITY: ORTHOPEDICS

DISCHARGE DIAGNOSIS :

ACL TEAR RIGHT KNEE

CO-MORBITIES WITH DURATION AND TREATMENT :

NIL

CHIEF COMPLAINTS:

C/O Pain in right knee, stiffness & difficulty in walking

BRIEF HISTORY AND EXAMINATION FINDINGS ON ADMISSION:

A 26 years old male came with above mentioned complaints. Patient was admitted here for further evaluation & management .

KEY EXAMINATION FINDING AT ADMISSION :

Patient is Conscious / Coherent / Oriented.
No Icterus / Clubbing / Cyanosis / Pallor / Pedal Edema.
TEMP :98.0 F
Pulse : 78/min
BP : 120/80mm Hg
RR : 20/min
JVP : Normal
CVS : S1,S2 +
SPO2 - 98% IN R/A

COURSE IN HOSPITAL (INCLUDING ALL REFERRAL CONSULTANT CLINICAL EXAMINATION AND OPINIONS):

Patient was admitted with above mentioned complaints . All relevant investigations were done. . After PAC and proper

SHREE USHA Diagnostics
Scientific Minds with Caring Hearts

Patient ID: 041125047
Patient Name: Mr. Sachin Mohapatra
Study Date: 04/11/2025

Age: 25 years
Sex: M
Referring Physician: Dr. Jeetendra Chekuri

MRI RIGHT KNEE

Clinical Profile: H/o twist injury.

Technique: Multiplanar mutiecho MRI of right knee has been performed using PDFS , T1W Sag and gradient sag sequences.

Findings:

- **Discontinuity noted in anterior cruciate ligament suggestive of complete tear.**
- **Buckling of posterior cruciate ligament noted.**
- **Altered signal intensity seen in anterior root of lateral meniscus suggestive of sprain.**
- **Altered signal intensity noted in joint capsule at posterolateral corner of knee joint with adjacent collection suggestive of grade I tear.**
- **T2 W1/PD FS hyperintensity noted in posterolateral aspect of upper end of tibia suggestive of contusion.**
- **Minimal joint space collection seen.**
- **Minimal prefemoral bursal collection seen.**
- The visualized osseous elements are intact with no evidence of fracture or dislocation. There is no evidence of osteochondral defect. There is no attenuation of articular cartilage.
- The medial meniscus is normal in appearance.
- The medial and lateral collateral ligaments and ligamentous complexes are intact.
- The patellar retinaculum, quadriceps tendon complex and the infrapatellar tendon are intact with no evidence of signal abnormality.
- The popliteal fossa is unremarkable.
- There is no Baker's cyst.

Impression: MRI – Right Knee reveals,

- **Complete tear in anterior cruciate ligament.**
- **Sprain in anterior root of lateral meniscus.**
- **Grade I tear in joint capsule at posterolateral corner of knee joint with adjacent collection.**
- **Contusion in posterolateral aspect of upper end of tibia.**
- **Minimal joint space collection.**
- **Minimal prefemoral bursal collection.**

DISCLAIMER: All modern machines/procedures have their own limitation. On arise of any clinical discrepancy, this investigation may be repeated or reassessed. This document is not subject to medicolegal purpose.

DR. D. TRIPATHY
MBBS, M.D.
CONSULTANT RADIOLOGIST

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Shree Hospitals

Hands that Serve • Heart that Care
A Unit of Odisha Multispeciality Hospital & Trauma Care Centre Pvt. Ltd.
ISO 9001 : 2015 Certified

17276

NABH

Date : 1/11/25

Patient Name : Sachin mahapatra Age : 25yr Sex : M

Dr. C. JEEVANDEEP Reg. No.

PR :	BPM
BP :	mmHg
RR :	bpm
SPO2 :	%RA
Temp. :	°F
CBG :	mg/dl
GCS :	/15
CNS :	
CVS :	
Chest :	
P/A :	

Pt had A/H/O fall while playing football at on
[illegible] & sustained injury to (R) knee &
C/O pain, swelling & instability.

No H/O headache, loc, vomiting, ENT bleed.

No H/O T.B, DM, HTN, Thyroid

O/E (R) knee

- Swelling - ~~[illegible]~~ minimal
- Tenderness +ve
- ROM - painful & restricted
- Ant drawer test +ve
- Lachman test +ve

Adv
- Ice pack Application
- Long knee brace to use
- Limb to be kept elevated.

Advice :

CBC
Na+ K+
Ur-, Cr-
LFT
RBS
HH
Urine RM
CXR - PA
ECG

MRI (R) knee

(1) T. ZEROFOL SP BD A/F x 5d
(2) T. PAN 40 OD B/F x 5d
(3) T. TENDOFAST FORTE OD A/F x 30d
(4) T. CALMOPLEX FORTE TDS A/F x 5d
(5) T. XM D 60K once weekly x 4 weeks

- Review c̄ Report.

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consent patient had undergone surgery as "ACL RECONSTRUCTION (RIGHT) KNEE " was done on 21/01/2026. Post operative period was uneventful. Patient was managed with IV fluids, antibiotics, analgesics, PPI and other supportive medication. Gradually patient improved & vital is stable. Hence he is discharged with the following medications & advice.

SURGERIES/PROCEDURE PERFORMED (Chronologically)

SURGERY DATE: 21/01/2026

NAME OF THE SURGERY /PROCEDURE: "ACL RECONSTRUCTION (RIGHT) KNEE "

ANAESTHESIA DETAILS:

TYPE OF ANAESTHESIA: SA + RA

NAME OF ANAESTHETIST: DR R , ROY

Condition at discharge in detail:
STABLE.

Discharge physiotherapy plan:
PHYSIOTHERAPY AS PER ADVISE.

Discharge medication including inputs of all referral doctors, continuation of earlier regular medication,
1.No abbreviations
2.Prescription in uppercase with name of drug, form, dose, frequency, duration, route, relation to food)

NAME OF THE DRUG	DOSE	ROUTE	FREQUENCY	DURATION	FREQUENCY QUALIFIER
TAB PANTOP DSR 40 MG	1 TAB	ORAL	ONCE DAILY	AT 7 AM X 30 DAYS	BEFORE BREAKFAST
TAB CEFTUM 500MG	1 TAB	ORAL	TWICE DAILY	AT 10 AM -10 PM X 7 DAYS	AFTER MEALS

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After 7 days then contin if pain

TAB ULTRACET	1TAB	ORAL	TWICE DAILY	AT 10AM -10PM X 7 DAYS &SOS	AFTER MEAL
SYP DUPHALAC	30 ML	ORAL	ONC EDAILY	AT 10 PM IF CONSTIPATION	AT BEDTIME
TAB RESNER - NT + 1M	1 TAB	ORAL	ONCE DAILY	AT 9 PM X 30 DAYS	AFTER MEAL
TAB ALPRAX 0.5MG	1TAB	ORAL	ONCE DAILY	AT 10 PM X 10 DAYS & SOS	AT BEDTIME

• celebrex 200 mg 1 tab SOS

POST DISCHARGE INSTRUCTIONS:
PHYSIOTHERAPY & MOBILIZATION 6 MONTHS
DIET AS PER DIETCIAN'S ADVICE.
REST AT HOME 4 WEEKS
DR TAPAS MITRA (PHYSIOTHERAPIST) PHONE NO- 9938587823

Follow up plan with date, any investigations and with whom:

Review after 12 weeks in Orthopedic OPD AT 3-4 PM.

"DO NOT STOP / START ANY DRUGS WITHOUT INFORMING THE DOCTOR".

When to obtain urgent care :- Fever, Pain, Bleeding or Discharge / Soak-age from surgical wound or any other compla contact the following given numbers.

CHECK LIST : # Drugs & Diet explained to patient. # Follow up visit to be explained (excluding holidays).

How to obtain urgent care: – (0674) 6165656

* For customer care number : ecare.carehospitals.com (0674 - 6165656)